

[illegible]

1. Standard regimen: 2 HRZE + 4 HR

WHAT YOU SEE

Sign '2 HRZE & 4 HR'

WHAT IT ENCODES

Drug-susceptible active TB: 2-month intensive phase with Rifampin, Isoniazid, Pyrazinamide, Ethambutol, then 4-month continuation with Rifampin + Isoniazid (total 6 months).

BOARD TRAP

Don't drop to 2 drugs in the intensive phase — 4 drugs prevent resistance until susceptibilities return.

2. Hepatotoxicity stop rule

WHAT YOU SEE

'STOP IF AST/ALT >3x ULN (symptomatic) or >5x ULN (asymptomatic)'

WHAT IT ENCODES

INH, RIF, and PZA are all hepatotoxic. Stop all if ALT/AST >3x ULN with symptoms or >5x ULN without symptoms, then reintroduce one drug at a time.

BOARD TRAP

Asymptomatic mild transaminase rise does NOT require stopping — threshold is higher (5x vs 3x) when asymptomatic.

3. Reintroduce one at a time / Rifampin first

WHAT YOU SEE

'REINTRODUCE ONE AT A TIME', 'RIFAMPIN FIRST (LEAST HEPATOTOXIC)'

WHAT IT ENCODES

After drug-induced hepatitis resolves, rechallenge sequentially. Rifampin is reintroduced first because it is the least hepatotoxic of the three.

BOARD TRAP

Restarting all drugs at once hides which agent caused hepatitis — reintroduce singly.

4. Isoniazid → B6 / peripheral neuropathy

WHAT YOU SEE

'B6', 'PYRIDOXINE 10–20 mg', 'GLOVES — PERIPHERAL NEUROPATHY'

WHAT IT ENCODES

INH causes pyridoxine (B6) deficiency → peripheral neuropathy. Give prophylactic B6 to those at risk: pregnant, diabetic, alcoholic, malnourished, HIV, uremic.

BOARD TRAP

Neuropathy is treated/prevented with B6, not by stopping INH; INH also causes drug-induced LUPUS and hepatitis.

5. INH drug-induced lupus

WHAT YOU SEE

'DRUG-INDUCED LUPUS', butterfly pin

WHAT IT ENCODES

INH is a classic cause of drug-induced lupus (anti-histone antibodies), along with procainamide and hydralazine.

BOARD TRAP

Drug-induced lupus from INH spares kidneys/CNS and is anti-histone positive; it resolves on stopping the drug.

6. Rifampin = CYP inducer

WHAT YOU SEE

Clock 'CYP INDUCER — DRUG INTERACTIONS'

WHAT IT ENCODES

Rifampin is a potent CYP450 INDUCER, lowering levels of warfarin, OCPs, protease inhibitors, methadone. It also causes orange body-fluid discoloration and thrombocytopenia.

BOARD TRAP

Rifampin INDUCES (not inhibits) CYP — it speeds metabolism and lowers drug levels; INH is the CYP inhibitor.

7. Rifampin → orange secretions

WHAT YOU SEE

'ORANGE BODY FLUID DISCOLORATION', 'ORANGE INK-STAINED'

WHAT IT ENCODES

Rifampin harmlessly turns urine, sweat, and tears orange-red and can permanently stain soft contact lenses.

BOARD TRAP

Orange fluids are benign and expected — not a reason to stop therapy; reassure the patient.

8. Pyrazinamide most hepatotoxic

WHAT YOU SEE

'MOST HEPATOTOXIC'

WHAT IT ENCODES

PZA is the most hepatotoxic first-line agent and also causes hyperuricemia/arthritis. It works in acidic intracellular/caseous environments.

BOARD TRAP

PZA-related arthritis/hyperuricemia is usually NOT true gout and you continue the drug; stop only for true hepatotoxicity.

9. PZA hyperuricemia → continue

WHAT YOU SEE

'ARTHRALGIA + HYPERURICEMIA → CONTINUE'

WHAT IT ENCODES

PZA inhibits renal urate excretion causing hyperuricemia and arthralgias; this is expected and you continue therapy.

BOARD TRAP

Asymptomatic hyperuricemia on PZA does NOT need treatment or drug cessation.

10. PZA avoided in pregnancy

WHAT YOU SEE

'PYRAZINAMIDE AVOIDED IN PREGNANCY'

WHAT IT ENCODES

In US practice PZA is often avoided in pregnancy (safety data limited); the pregnancy regimen is RIF + INH + EMB for 9 months. INH, RIF, EMB are considered safe.

BOARD TRAP

Streptomycin is the agent absolutely contraindicated in pregnancy (ototoxic to fetus), not the oral first-line drugs.

11. Ethambutol → optic neuritis

WHAT YOU SEE

'PYRAZINAMIDE VISUAL CHANGES → STOP PERMANENTLY', 'MONTHLY VISUAL ACUITY CHECK'

WHAT IT ENCODES

Ethambutol causes dose-dependent optic neuritis with red-green color blindness and decreased acuity. Check visual acuity/color monthly and STOP permanently if it occurs.

BOARD TRAP

Optic neuritis is the EMB (ethambutol) side effect — the panel's label is a deliberate distractor; EMB toxicity is irreversible-risk so stop, don't rechallenge.

12. MDR-TB / BPaLM regimen

WHAT YOU SEE

'BPaLM 6 MONTHS — MDR/RR-TB', 'XDR-TB'

WHAT IT ENCODES

MDR-TB (resistant to INH + RIF) is treated with the 6-month BPaLM regimen: Bedaquiline, Pretomanid, Linezolid, Moxifloxacin. XDR adds fluoroquinolone/second-line injectable resistance.

BOARD TRAP

Bedaquiline and moxifloxacin prolong QT — BPaLM is contraindicated if QTc >500 ms.

13. QTc >500 ms contraindicated

WHAT YOU SEE

'QTc >500ms → CONTRAINDICATED'

WHAT IT ENCODES

Bedaquiline (and moxifloxacin) cause QT prolongation; monitor ECG and avoid initiating if QTc exceeds 500 ms or with other QT-prolonging drugs.

BOARD TRAP

This QT rule is for the MDR (BPaLM) drugs, not first-line RIPE.

14. LTBI regimens: 3HP / 4R / INH

WHAT YOU SEE

'INH 9 MONTHS', '3HP 12 weekly INH+rifapentine', '4R rifampin 4 months'

WHAT IT ENCODES

Latent TB options: 3HP (weekly INH+rifapentine ×12, preferred), 4R (rifampin ×4 months), or INH ×9 months. Shorter rifamycin-based regimens improve completion.

BOARD TRAP

Exclude ACTIVE TB before starting LTBI monotherapy — treating active disease with one drug breeds resistance.

15. Exclude active TB before TPT

WHAT YOU SEE

'EXCLUDE ACTIVE TB FIRST → TPT = MONOTHERAPY = RESISTANCE'

WHAT IT ENCODES

Always rule out active TB (symptoms, CXR, sputum) before starting single/dual-drug preventive therapy; otherwise you select for resistance.

BOARD TRAP

Never give LTBI monotherapy to someone with undiagnosed active disease.